SOMATIC SYMPTOM & RELATED DISORDERS

Somatic symptom disorders, as described in the **DSM-5**, are a group of conditions that lie at the intersection of psychology and medicine. They involve **physical (bodily) symptoms along with abnormal thoughts, feelings, and behaviours related to those symptoms**. The term "somatic" itself means body, and in these disorders individuals genuinely experience distressing physical complaints—such as pain, fatigue, or paralysis—that suggest a medical condition. However, these symptoms are often **not fully explained by any identifiable medical cause**, or if a medical condition exists, the person's reaction to it is **excessive and disproportionate**.

Key DSM-5 Shift

- Earlier (DSM-IV): Focus on **"medically unexplained symptoms"**
- Now (DSM-5): Focus on **maladaptive response to symptoms**, not whether they are medically explained

The focus now is on the **person's psychological response**, their persistent worry, preoccupation, and impairment in daily functioning.
Individuals with these disorders tend to be **highly focused on their health**, often believing that something is seriously wrong despite reassurance. Importantly, they are **not consciously faking or producing symptoms**; their distress is real, and they typically seek frequent medical help.

DSM-5 includes four main disorders in this category.

1. **Somatic Symptom Disorder** involves one or more physical symptoms accompanied by excessive thoughts and anxiety about health.
2. **Illness Anxiety Disorder or Hypochondriasis** is characterized by a strong fear of having a serious illness despite minimal or no symptoms.
3. **Conversion Disorder** presents with neurological symptoms like paralysis or blindness that cannot be medically explained and are linked to psychological factors. Finally,
4. **Factitious Disorder** differs in that symptoms are **intentionally produced**, but the goal is to assume the "sick role" rather than gain external rewards.

This category is distinct from **malingering**, where symptoms are deliberately faked for clear external benefits such as avoiding responsibilities. In contrast, somatic symptom disorders primarily involve **genuine distress, lack of conscious control over symptoms, and significant impairment in functioning due to health-related preoccupation**.

ILLNESS ANXIETY DISORDER (HYPOCHONDRIASIS)

Previously known as **hypochondriasis**, this condition is now termed **Illness Anxiety Disorder** in DSM-5.

It is characterized by a **persistent preoccupation with having or developing a serious illness**, even when there are **no or only mild physical symptoms**. The concern arises mainly due to **misinterpretation of normal bodily sensations**.

criteria for
Illness Anxiety Disorder

DSM-5

A. Preoccupation with having or acquiring a serious illness.

B. Somatic symptoms are not present or, if present, are only mild in intensity. If another medical condition is present or there is a high risk for developing a medical condition (e.g., strong family history is present), the preoccupation is clearly excessive or disproportionate.

C. There is a high level of anxiety about health, and the individual is easily alarmed about personal health status.

D. The individual performs excessive health-related behaviors (e.g., repeatedly checks his or her body for signs of illness) or exhibits maladaptive avoidance (e.g., avoids doctor appointments and hospitals).

E. Illness preoccupation has been present for at least 6 months, but the specific illness that is feared may change over that period of time.

F. The illness-related preoccupation is not better explained by another mental disorder, such as somatic symptom disorder, panic disorder, generalized anxiety disorder, body dysmorphic disorder, obsessive-compulsive disorder, or delusional disorder, somatic type.

Source: Reprinted with permission from the *Diagnostic and Statistical Manual of Mental Disorders*, Fifth Edition, (Copyright 2013). American Psychiatric Association.

Key Characteristics

- Constant fear or belief of having a **serious disease**
- Misinterpretation of minor bodily sensations
  - e.g., slight cough → lung cancer
- **Not reassured by medical evaluation**
- Repeated:
  - doctor visits or
  - doctor shopping
- High **health anxiety** and preoccupation
- Symptoms persist for **at least 6 months**
- Causes **significant distress and impairment**
- Insight is present (belief is not delusional)

In DSM-5: Focus is on **health anxiety** rather than physical symptoms. Physical symptoms are usually **minimal or absent**

Behavioral Pattern

- Frequently visit doctors
- Doubt medical opinions (“something was missed”)
- Spend excessive time researching symptoms
- Resist psychological explanations

This leads to: High medical costs & Chronic worry

Causes (Cognitive-Behavioral View)

The most accepted explanation is **cognitive-behavioral**:

1. Dysfunctional beliefs about health

This is developed through: Personal illness experiences, Family illness history, Media exposure

Examples:
- "All bodily changes indicate serious illness"
- "If I don't act immediately, it will be too late"

2. Attentional bias: There is excessive focus on bodily sensations and Heightened awareness of minor changes

3. Misinterpretation of sensations: Normal sensations are perceived as **dangerous and** Overestimation of illness severity

4. Confirmation bias: Seek evidence that supports illness belief and Ignore reassurance or normal results

5. Low perceived coping ability: Belief that they cannot handle illness and View themselves as physically weak

Mechanism of Cause

1. **Trigger**: Minor bodily sensation
2. **Cognitive distortion**: Catastrophic interpretation
3. **Anxiety**: Fear of illness increases
4. **Physical symptoms**: Anxiety produces real sensations (e.g., palpitations)
5. **Reinforcement**: Symptoms confirm fear

Vicious Cycle: Sensation → Misinterpretation → Anxiety → More symptoms → Stronger belief

Role of Learning & Reinforcement: These reinforce illness-focused behavior

- Childhood experiences: Frequent illness & Attention during sickness
- Family environment: Exposure to illness behavior
- Secondary gains: Attention & Relief from responsibilities

Treatment- Cognitive Behavioral Therapy (CBT) is most effective:

- **Cognitive component**:
    - Challenge irrational beliefs
    - Correct misinterpretations
- **Behavioral component**:

- Reduce checking behaviors
- Limit reassurance-seeking
- Exposure to bodily sensations

Typically brief (6–16 sessions) and effective in reducing: Anxiety, Illness beliefs & Symptom preoccupation.

CONVERSION DISORDER/ FUNCTIONAL NEUROLOGICAL SYMPTOM DISORDER

Conversion disorder is a condition in which a person shows **neurological symptoms affecting motor or sensory functions** (such as paralysis or blindness) that **appear medical**, but **cannot be explained by any known medical condition**.

A key feature is that **psychological factors (stress or conflict)** play an important role in the onset or worsening of symptoms, and the person is **not intentionally faking them**.

criteria for
Conversion Disorder

DSM-5

A. One or more symptoms of altered voluntary motor or sensory function.

B. Clinical findings provide evidence of incompatibility between the symptom and recognized neurological or medical conditions.

C. The symptom or deficit is not better explained by another medical or mental disorder.

D. The symptom or deficit causes clinically significant distress or impairment in social, occupational, or other important areas of functioning or warrants medical evaluation.

Source: Reprinted with permission from the *Diagnostic and Statistical Manual of Mental Disorders*, Fifth Edition, (Copyright 2013). American Psychiatric Association.

Key Characteristics

- Symptoms mimic neurological disorders:
  - Paralysis
  - Blindness
  - Deafness
  - Pseudoseizures
- Symptoms are **inconsistent with medical knowledge**
- Often begin after **stressful or traumatic events**
- Person has **no conscious control** over symptoms
- May show **la belle indifférence** (lack of concern), though not always
- Causes **distress or functional impairment**

Historical Background (Freud's View)

Previously called **"conversion hysteria",** Freud believed that Symptoms arise from **repressed unconscious conflicts** (often sexual) and Anxiety is **converted into physical symptoms**

Example: Guilt about a thought → paralysis of hand

Gains:

- **Primary gain**: Reduction of internal anxiety/conflict
- **Secondary gain**: External benefits (attention, sympathy, escape)

While Freud's exact ideas are not fully accepted today, the concepts of **primary and secondary gain are still important**

Mechanism of Cause (Modern View)

1. Stressful or traumatic event: During a stressful event a person faces a situation that is Intolerable and Inescapable

2. Psychological conflict: Desire to escape but cannot do so openly

3. Unconscious conversion: Stress is **converted into physical symptoms** and there is No conscious awareness of this process

4. Functional disruption: Here, Brain functioning (not structure) is altered and Sensory/motor signals are received, but blocked from conscious awareness

5. Reinforcement

- **Primary gain**: Avoidance of stress
- **Secondary gain**:
    - Attention
    - Sympathy
    - Relief from responsibilities

Stress → Conflict → Unconscious conversion → Physical symptom → Reinforcement

Precipitating Factors: Emotional conflicts, Interpersonal problems, Trauma or stress, High-impact life events
Greater stress → more severe symptoms

Types of Symptoms

1. Sensory Symptoms: Sensory input is **registered but not consciously perceived** causing Blindness, Deafness, Loss of touch (anesthesia)

Example: "Glove anesthesia" (loss of sensation in hand not matching anatomy)

2. Motor Symptoms: Function may appear selectively impaired causing Paralysis (often one limb), Inability to speak (aphonia), Difficulty swallowing (globus sensation)

Example: cannot walk normally but can run in danger

3. Seizures (Pseudoseizures): Resemble epileptic seizures, But show No EEG abnormalities nor No post-seizure confusion. It often involves dramatic movements without injury

4. Mixed Symptoms: Combination of sensory + motor + seizure symptoms

Course and Demographics

- More common in **women**
- Usually begins in Adolescence or early adulthood
- Often has **sudden onset after stress**
- May resolve quickly if stress is removed Or become chronic and recurrent

🔹 **Important Clinical Insight**

- Symptoms provide a **"safe escape"** from stress
- Especially when direct escape is not possible or Socially unacceptable

Example: Soldiers developing paralysis to avoid combat without being labeled cowardly

A person facing severe family conflict suddenly loses the ability to walk, even though medical tests show no neurological damage.

Treatment

1. Behavioral Therapy: Encourage normal movement, Reinforce recovery (praise, privileges), Remove reinforcement for symptoms

2. Cognitive Behavioral Therapy (CBT): Address stress and maladaptive thinking, Particularly useful for pseudoseizures

3. Other Approaches

- Hypnosis (in some cases)
- Stress management

FACTITIOUS DISORDER

condition in which a person **deliberately produces, fakes, or exaggerates physical or psychological symptoms** in order to assume the "sick role" and gain attention, care, and sympathy. Unlike somatic symptom disorders, the symptoms here are **intentional**, but unlike malingering, there are **no clear external rewards** such as money or avoiding responsibilities, the motivation is psychological. Individuals may lie about symptoms, tamper with medical tests, or even harm themselves to appear ill, and they often frequently visit hospitals. The disorder is thought to arise from **deep emotional needs, low self-esteem, or past experiences of neglect or illness**, and is maintained because the attention and care received reinforce the behavior.

DISSOCIATIVE DISORDERS

Dissociative disorders are a group of conditions involving **disruptions in the normal integration of consciousness, memory, identity, and perception**. In these disorders, the normally coordinated mental processes become **disconnected**, leading to unusual experiences such as **loss of memory, identity confusion, or multiple identities**.

The concept of **dissociation** refers to the mind's ability to split off certain mental processes from conscious awareness. Mild dissociation is common (e.g., daydreaming, losing track while driving), but in these disorders, dissociation becomes **extreme and maladaptive**, causing significant distress and impairment.

DEPERSONALIZATION/DEREALIZATION DISORDER

Dissociative condition in which a person experiences **persistent or recurrent feelings of detachment** either from **their own self (depersonalization)** or from **the external world (derealization)**.

While mild forms of these experiences are common (especially during stress or sleep deprivation), the disorder is diagnosed when these experiences become **chronic, distressing, and interfere with daily functioning**.

Depersonalization Vs Derealization

- **Depersonalization**: Feeling detached from oneself
    - As if observing oneself from outside
    - Feeling unreal, changed, or disconnected from one's body
- **Derealization**: Feeling detached from the external world
    - Surroundings appear strange, dreamlike, or artificial
    - People and objects seem lifeless or mechanical

Many individuals experience **both together**

Key Characteristics

- Sense of **detachment from self or surroundings**
- Feeling like:
    - "living in a dream or movie"
    - being an outside observer
- Self or world appears:
    - unreal
    - unfamiliar
    - mechanical ("like automatons")
- **Reality testing remains intact**
    - person knows the experience is not real (unlike psychosis)
- **Reduced emotional responsiveness**
- Time distortion and **fragmented memory**

Cognitive and Emotional Features

- Emotional experiences are **blunted or reduced**
- Difficulty forming **coherent memory of events** (memory fragmentation)
- Altered sense of time
- Experiences feel **discontinuous from normal self**

Causes / Mechanism

1. Stress and Overload: Often triggered by severe stress, trauma, sleep deprivation

2. Dissociation as a Defense: Mind "disconnects" from overwhelming emotions & distressing experiences

3. Emotional Numbing: Brain reduces emotional responses which leads to detachment and unreality feeling

Stress → Dissociation → Emotional numbing → Detachment from self/world

Course

- Average onset: early adulthood (~23 years)
- Often **chronic (around 80% cases)**
- Symptoms may remain stable over time

Example: A person feels as if they are **watching themselves from outside their body**, and the world around them appears **foggy and unreal**, yet they know logically that nothing has actually changed.

DISSOCIATIVE / PSYCHOGENIC AMNESIA

Dissociative amnesia is a disorder in which a person is **unable to recall important autobiographical information**, usually related to **trauma or stress**, and this cannot be explained by normal forgetting.

It occurs because the normally integrated mental processes become **disrupted**, making certain memories **inaccessible to conscious awareness**.

Nature of Memory Loss

- Mainly **retrograde amnesia**:
  - loss of past personal memories
- Differs from brain injury:
  - brain damage → **anterograde amnesia** (cannot form new memories)
  - dissociative amnesia → past memories are **stored but inaccessible**

The forgotten information still exists and may return spontaneously or under hypnosis or narcosis.

Causes: Triggered by **intolerable stress or trauma**, such as:

- war experiences
- serious accidents
- suicide attempts
- violent events

Acts as a **psychological defense** to block distressing memories

Types

- **Localized amnesia**: no memory for a specific period (often after trauma)
- **Selective amnesia**: partial memory of an event

Key Characteristics

- Loss of autobiographical memory
- No neurological cause
- Other functions remain intact:
  - language
  - skills
  - intelligence
- Causes **distress or impairment**

Example: A person involved in a severe accident cannot remember the incident or events surrounding it.

DISSOCIATIVE FUGUE

Dissociative fugue is a **rare and more extreme form of dissociative amnesia**, where memory loss is accompanied by **sudden travel or wandering away from home**.

Key Features

- Amnesia for:
  - personal identity
  - past life
- **Sudden departure ("flight")** from home
- Confusion about identity or adoption of a **new identity**
- Behavior appears **normal during fugue**

- New lifestyle may differ from previous one

Memory Pattern

- During fugue: unaware of past identity, memory for current events is intact
- After recovery: original memory returns, **amnesia for fugue period occurs**

Course

- May last: days, weeks, months or even years
- Recovery may be sudden or gradual with reminders

Example: A person leaves home, starts living in another city under a new identity, and later suddenly realizes they do not remember how they got there.

Relationship Between Amnesia and Fugue

- Dissociative fugue is considered a **subtype of dissociative amnesia**
- Both involve:
    - memory loss due to stress
- Fugue includes an additional feature:
    - **physical escape (travel) from the situation**

DISSOCIATIVE IDENTITY DISORDER

Dissociative Identity Disorder (DID), formerly called multiple personality disorder, is a severe dissociative condition in which a person shows **two or more distinct identities (alter identities)** that **alternately take control of behavior**. It is also marked by **gaps in memory** that cannot be explained by ordinary forgetting.

Importantly, DID reflects a **failure to integrate different aspects of identity, memory, and consciousness**, rather than the presence of fully separate personalities.

Key Characteristics

1. Multiple Identities (Alters)

- Two or more distinct identity states
- Each identity may have: its own name, personal history, self-image
- Differences between alters can include: gender, age, handwriting, language, behavior and emotions

Example: One alter may be outgoing and carefree, while another is serious and reserved.

2. Host Identity

- The **host identity**: is most frequently present and carries the person's real name
- It is **not always the original identity**
- May not be the best-adjusted identity

3. Switching Between Identities

- Alters take control **at different times**
- Switching can be sudden (within seconds) or gradual
- Sometimes triggered by stress or internal cues

4. Memory Gaps (Amnesia)

- Inability to recall: everyday events, personal information, actions of other identities
- Amnesia may be **uneven**: some identities know about others and others remain unaware

5. Lack of Integration

- Core problem: **identity, memory, and consciousness are not integrated**
- As noted by theorists: the issue is not having multiple personalities but **a fragmented or incomplete sense of self**

Additional Symptoms: Depression, Self-harm (self-mutilation), Suicidal thoughts or attempts, Erratic behavior, Headaches, Hallucinations (e.g., voices inside the head), Post-traumatic symptoms, Amnesia and fugue-like episodes

Course and Demographics

- Usually **begins in childhood**
- Often diagnosed later: teens, 20s, or 30s
- More common in females: about **3–9 times higher than males**

Causes

- Strongly linked to **severe childhood trauma**, especially abuse
- Dissociation acts as a **coping mechanism** different identities handle different emotions or experiences

Example: A person alternates between different identities—one may be confident and outgoing, while another is fearful and withdrawn—with **no memory of what happened in the other state**.

CAUSES OF DISSOCIATIVE DISORDERS

Dissociative disorders arise due to a combination of **biological, psychological, and sociocultural factors**, often interacting with each other.

Biological Causes

- **Neurobiological abnormalities**:
  Brain imaging studies suggest differences in areas involved in memory and emotion, such as:
  - hippocampus (memory)
  - amygdala (emotion)
  - orbitofrontal cortex (decision-making)
- **Stress response dysfunction**:
  Dysregulation of the **HPA axis (hypothalamic–pituitary–adrenal system)** may lead to an abnormal response to trauma, increasing dissociation.

Psychological Factors

- **Trauma history (most important factor)**:
  - Severe trauma, especially **childhood abuse** (physical, emotional, sexual)
  - Dissociation acts as a **coping mechanism**, helping the person detach from overwhelming experiences
- **Psychodynamic perspective**:
  - Dissociation functions as a **defense mechanism**
  - Keeps painful thoughts and memories **out of conscious awareness**

Sociocultural Factors

- **Cultural influences**:
  - Some cultures accept trance or possession states as normal expressions of distress
- **Media influence**:
  - Portrayals in movies/books may shape how symptoms are expressed, especially in DID
- **Social reinforcement** (controversial):
  - Symptoms may be maintained by:
    - attention
    - escape from responsibility

Treatment of Dissociative Disorders

Treatment focuses mainly on **psychotherapy and trauma resolution**, rather than medication.

Psychotherapy

1. Trauma-Focused Therapy

- Core treatment approach
- Helps process past trauma
- Includes:
    - psychodynamic therapy
    - cognitive behavioral therapy (CBT)
    - EMDR (Eye Movement Desensitization and Reprocessing)

2. Integration of Identities (for DID)

- Goal: combine different identities into **one cohesive self**
- Focus on:
    - building trust
    - resolving trauma
    - improving coping skills

3. Grounding Techniques

- Especially useful for:
    - depersonalization/derealization
- Helps individuals:
    - stay present
    - reconnect with reality

Medication

- No specific drug cures dissociative disorders
- Used only for **associated symptoms**:
    - Antidepressants (SSRIs) → depression/anxiety
    - Anxiolytics → anxiety (used cautiously)
    - Antipsychotics → rarely, as adjunct

Supportive Interventions

- **Psychoeducation**:
    - Educating patients and families
    - Reduces stigma and improves understanding
- **Crisis intervention**:
    - Needed during:
        - severe distress
        - suicidal risk